Humana. Claims Payment Policy

Subject: Neonatal Intensive Care Unit (NICU) Guidelines

Application: Florida Medicaid, Kentucky Medicaid, Oklahoma Medicaid, South Carolina Medicaid and Virginia Medicaid Products
Policy number: CP2022002
Related policies: N/A

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Overview

The Neonatal Intensive Care Unit (NICU) provides continuous care to a critically ill neonate and involves many advanced technologies and interventions that are unique to the specialty area. These critically ill neonates can suffer from various medical conditions including congenital heart defects, neural tube defects, prematurity, respiratory distress, metabolic disorders and difficulties with feeding and growth.

Diagnosis related groups (DRGs), including Medicare Severity Diagnosis Related Groups (MS-DRGs) and All Patient Refined Diagnosis Related Groups (APR-DRGs), are assigned to newborns based on factors including, but not limited to, birth weight, extent of prematurity, diagnoses, severity of illness, and associated significant problems.

NICU is comprised of four levels of care: Level 1, Level 2, Level 3 and Level 4. Each of the four levels of care has a corresponding revenue code: 0171, 0172, 0173 and 0174, respectively.

This policy outlines Humana's billing requirements and reimbursement for NICU claims.

Please note the definitions and background information for the following terms that are relevant for this policy:

- Neonate: An infant who is less than four weeks old.
- Prematurity: The condition when a neonate is born before a gestational age of 37 weeks and zero days.
- Extreme prematurity: The condition when a neonate is born before a gestational age of 28 weeks and zero days.
- Neonatal respiratory distress syndrome (RDS) (ICD-10-CM diagnosis code P22.0): Occurs when there is a deficiency of surfactant. This is primarily due to insufficient production of surfactant or in rare cases, it can be caused by a genetic mutation in surfactant proteins or the transporter genes. Prematurity is the primary cause as surfactant production by the lungs does not occur until around a gestational age of 26 weeks, so the majority of cases are seen in the extreme premature population. In total, all case-types of newborn respiratory distress occurs in approximately 7 percent of deliveries with only about 1 percent resulting in RDS. Prematurity and low birth weight are the most important risk factors in the development of RDS and usually occurs before a gestational age of 28 weeks. Chest X-ray findings for RDS are classic. The findings include diffuse atelectasis, with a ground-glass appearance, air bronchograms and low lung volumes. Treatment includes surfactant therapy; assisted ventilation with a ventilator, continuous positive airway pressure (regular or nasal), or other assisted ventilation such as high flow oxygen; monitoring of oxygenation; and supportive care (thermoregulation, nutritional support, fluid and electrolyte management, etc.)
- Transient tachypnea of the newborn (ICD-10-CM diagnosis code P22.1): A transient respiratory distress caused when fetal lung fluid that is not readily absorbed from the lungs during the delivery process. Chest X-ray findings show lungs that are normal or hyperinflated. There is noted perihilar streaking, and the lung fissures can demonstrate fluid in them. Treatment is oxygen and supportive care. This typically resolves within a few days.

- Other respiratory distress of newborn (ICD-10-CM diagnosis code P22.8): Respiratory distress that occurs in the neonatal period that is related to a specific disease process. Examples include respiratory distress secondary to pneumonia, sepsis, meconium aspiration, congenital heart defect, etc.
- Respiratory distress of newborn, unspecified (ICD-10-CM diagnosis code P22.9): Respiratory distress that occurs in the neonatal period that is not related to a specific identified disease process or does not meet the definition of transient tachypnea of the newborn or RDS.

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Virginia Medicaid Payment Policy
References
General Humana Resources

Florida Medicaid Payment Policy

In addition to the policy, claims payments are subject to other plan requirements for the processing and payment of claims, including, but not limited to, requirements of medical necessity and reasonableness and applicable referral or authorization requirements.

To be reimbursed for a medically necessary NICU stay subject to DRG-based pricing, the provider must satisfy the requirements in this policy.

During the NICU stay, the provider must provide all relevant documentation, such as medical records, for utilization management and leveling purposes. Humana uses all relevant documentation to assign the DRG for the case according to the table below. For a case that qualifies for more than one DRG, Humana assigns the DRG associated with the most complex services.

DRG code	Minimum criteria
580	A neonate transferred from any hospital who is less than 5 days old and not born at the current hospital
581	A neonate transferred from any hospital who is less than 5 days old and born at the current hospital
583	A neonate with extracorporeal membrane oxygenation (ECMO)
588	A neonate who has a birth weight less than 1500 grams with a major procedure
589	A neonate who has a birth weight less than 500 grams, or a birth weight of 500 grams to 999 grams and a gestational age of less than 24 weeks, or a birth weight of 500 grams to 749 grams with a major anomaly or without life sustaining intervention
591	A neonate who has a birth weight of 500 grams to 749 grams without a major procedure
593	A neonate who has a birth weight of 750 grams to 999 grams without a major procedure
602	A neonate who has a birth weight of 1000 grams to 1249 grams with RDS or another major respiratory condition or major anomaly
603	A neonate who has a birth weight of 1000 grams to 1249 grams with or without another significant condition
607	A neonate who has a birth weight of 1250 grams to 1499 grams with RDS or another major respiratory condition or major anomaly
608	A neonate who has a birth weight of 1250 grams to 1499 grams with or without other significant condition
609	A neonate who has a birth weight of 1500 grams to 2499 grams with a major procedure
611	A neonate who has a birth weight of 1500 grams to 1999 grams with a major anomaly
612	A neonate who has a birth weight of 1500 grams to 1999 grams with RDS or another major respiratory condition
613	A neonate who has a birth weight of 1500 grams to 1999 grams with a congenital or perinatal infection
614	A neonate who has a birth weight of 1500 grams to 1999 grams with or without another significant condition
621	A neonate who has a birth weight of 2000 grams to 2499 grams with a major anomaly
622	A neonate who has a birth weight of 2000 grams to 2499 grams with RDS or another major respiratory condition
623	A neonate who has a birth weight of 2000 grams to 2499 grams with a congenital or perinatal infection

DRG code	Minimum criteria
625	A neonate who has a birth weight of 2000 grams to 2499 grams with another significant condition
626	A normal newborn or neonate who has a birthweight of 2000 grams to 2499 grams with another problem
630	A neonate who has a birth weight greater than 2499 grams with a major cardiovascular procedure
631	A neonate who has a birth weight greater than 2499 grams with another major procedure
633	A neonate who has a birth weight greater than 2499 grams with a major anomaly
634	A neonate who has a birth weight greater than 2499 grams with RDS or another major respiratory condition
636	A neonate who has a birth weight greater than 2499 grams with a congenital or perinatal infection
639	A neonate who has a birth weight greater than 2499 grams with another significant condition
640	A normal newborn or neonate who has a birthweight greater than 2499 grams with another problem
863	Neonatal aftercare

Humana uses all relevant documentation to review the ICD-10-CM diagnosis codes reported on the claim according to the peer-reviewed literature definition and treatment requirements. If a reported diagnosis code does not satisfy the applicable requirement, it is not considered when calculating the DRG reimbursed.

Humana reviews the level of care as reported by the NICU revenue code according to the table below. If the documentation associated with a charge for a NICU revenue code does not establish that the criteria associated with that revenue code were satisfied, reimbursement for the claim will instead be calculated using the highest NICU revenue code for which the criteria are satisfied.

Nursery level	Revenue code	Minimum criteria
Level 0	0170	Level 0 and Level 1 are considered newborn nursery. Examples include care to observe for: • Sepsis • Temperature instability • Feeding irregularities • Jaundice
Level 1	0171	
Level 2	0172	Neonate who is not severely ill but needs more hours of nursing and medical care than typical neonates, as indicated by **one or more of the following**: • Ventilatory support using one of the following: – Hood oxygen, or – Less than or equal to two liters per minute of oxygen via nasal cannula • Administration of intravenous (IV) heplock medicines • Weaning from nasogastric (NG) or nasojejunal (NJ) tube feedings while trying to increase oral intake, without need for IV fluids • Weaning from isolette or radiant warmer to open crib, without need for temperature support • Documented episodes of apnea, bradycardia or desaturation, for example: – Apnea countdown, – Titrating caffeine or preparing infant for home caffeine administration, or

Nursery level	Revenue code	Minimum criteria
		– Preparing infant for home monitor use for apnea, bradycardia or desaturation • Services for neonatal abstinence syndrome (NAS) when the Finnegan score is less than or equal to 8 • Services for neonatal jaundice requiring either: – Use of phototherapy, or – Monitoring of bilirubin levels less frequently than every 12 hours • Supportive care for growing premature infant, or establishing weight gain
Level 3	0173	**Level 2 requirements are met** **and** Temperature control modalities, such as isolette or radiant warmer, are in use **and** Condition requiring 6 to 12 hours of active nursing and medical evaluation, treatment and support, as evidenced by **one or more of the following**: • Ventilatory support using one of the following: – Greater than two liters per minute of oxygen via nasal cannula, – High flow nasal cannula, – Continuous positive airway pressure (CPAP), – Stable chronic mechanical ventilation (on home ventilator settings), or – Conventional ventilator • Umbilical venous catheter (UVC) or invasive cardiopulmonary monitoring (CPM) • Hypoglycemia requiring D-10 bolus therapy • Nothing by mouth (NPO) with either: – All NG feedings, or – Total parenteral nutrition (TPN) • Services for NAS with one of the following: – Three consecutive Finnegan scores greater than or equal to 8, – Two consecutive Finnegan scores greater than or equal to 12, or – Pharmacologic management of NAS
Level 4	0174	**Level 3 requirements are met** **and** Other condition that requires ongoing evaluation, active management, or therapy adjustment with near continuous nursing and medical support, as evidenced by **one or more of the following**: • Ventilatory support using high frequency oscillating ventilator (HFOV) • Use of IV bolus therapy, inotropic drugs, chronotropic drugs, calcium channel blockers or IV prostaglandin therapy • Active seizures, with need for anticonvulsants • Surgical repair of severe congenital defect, for example: – Omphalocele repair, – Bowel resection for necrotizing enterocolitis (NEC), or – Tracheoesophageal fistula repair • Inhaled nitric oxide (iNO) • Extracorporeal membrane oxygenation (ECMO) • Therapeutic cooling • Hyperbilirubinemia requiring either: – Exchange transfusion, or – Intravenous immunoglobulin (IVIG) administration • Emergent event (for example, pulmonary arrest or cardiac arrest)

Kentucky Medicaid Payment Policy

In addition to the policy, claims payments are subject to other plan requirements for the processing and payment of claims, including, but not limited to, requirements of medical necessity and reasonableness and applicable referral or authorization requirements.

To be reimbursed for a medically necessary NICU stay subject to DRG-based pricing, the provider must satisfy the requirements in this policy.

During the NICU stay, the provider must provide all relevant documentation, such as medical records, for utilization management and leveling purposes. Humana uses all relevant documentation to assign the DRG for the case according to the table below. For a case that qualifies for more than one DRG as outlined in the table below, Humana assigns the DRG associated with the most complex services.

DRG code	Minimum criteria
790	A neonate who has a birth weight less than or equal to 999 grams, has completed a gestational age of 23 – 26 weeks or is diagnosed with neonatal RDS (ICD-10-CM diagnosis code P22.0) according to the peer-reviewed literature definition and treatment requirements noted in the Overview section ***Note:*** DRG 790 assignment is intended for critically ill neonates requiring the most complex care **and** prolonged hospital confinement.
791	A neonate who has a birth weight of 1,000 grams to 2,499 grams or has completed a gestational age of only 27 – 36 weeks **and** One or more of the diagnoses on the Centers for Medicare & Medicaid Services (CMS) Major Problems Diagnoses list applicable for the date of service, available in the References section
792	A neonate who has a birth weight of 1,000 grams to 2,499 grams or has completed a gestational age of only 27 – 36 weeks **without** major problems
793	A full-term neonate who has completed a gestational age of 37 weeks or greater **and** One or more of the diagnoses on the CMS Major Problems Diagnoses list, applicable for the date of service, available in the References section
794	A full-term neonate who has completed a gestational age of 37 weeks or greater **and** One or more of the diagnoses on the CMS Significant Problems Diagnoses list, applicable for the date of service, available in the References section
795	A normal neonate not requiring care outside of the newborn nursery
789	A neonate who is transferred to another acute care facility or has died

Humana uses all relevant documentation to review the ICD-10-CM diagnosis codes reported on the claim according to the peer-reviewed literature definition and treatment requirements. If a reported diagnosis code does not satisfy the applicable requirement, it is not considered when calculating the DRG reimbursed.

For a claim with a discharge date on or after May 29, 2025, Humana reviews additional factors to calculate reimbursement of the DRG outlier payment, when applicable, according to CMS MS-DRG guidelines. This includes reviewing whether the provider has billed for any days that do not meet medical necessity requirements. It also includes reviewing the following data on the claim:

- The level of care as reported by the NICU revenue code, according to the applicable criteria established by MCG Health;
- The number of days billed; and
- The dates of service.

Oklahoma Medicaid Payment Policy

In addition to the policy, claims payments are subject to other plan requirements for the processing and payment of claims, including, but not limited to, requirements of medical necessity and reasonableness and applicable referral or authorization requirements.

To be reimbursed for a medically necessary NICU stay subject to DRG-based pricing, the provider must satisfy the requirements in this policy.

Providers must follow all applicable Oklahoma Health Care Authority (OHCA) guidance for NICU claims outlined at OHCA's Payment for Newborns website, available in the References section, including but not limited to the following:

- Report the ICD-10-CM diagnosis code indicating the neonate's birth weight.

During the NICU stay, the provider must provide all relevant documentation, such as medical records, for utilization management and leveling purposes. Humana uses all relevant documentation to assign the DRG for the case according to the tables below. For a case that qualifies for more than one DRG, Humana assigns the DRG associated with the most complex services.

OHCA DRG code	Minimum criteria
N01	• The applicable MS-DRG 789 – 795 criteria were satisfied as outlined in the chart below; • The length of stay was less than two days; • The neonate died before discharge; **and** • The neonate was not transferred from another acute care facility.
N02	• The applicable MS-DRG 789 – 795 criteria were satisfied as outlined in the chart below; • The length of stay was less than two days; • The neonate died before discharge; **and** • The neonate was transferred from another acute care facility.
N04	• The applicable MS-DRG 789 – 795 criteria were satisfied as outlined in the chart below; • The length of stay was less than five days; **and** • The neonate was transferred to another acute care facility.
N10	• The applicable MS-DRG 789 – 795 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of less than 750 grams; **and** • The neonate died before discharge.
N14	• The applicable MS-DRG 789 – 795 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of less than 750 grams; **and** • The neonate did not die before discharge.
N20	• The applicable MS-DRG 789 – 795 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of 750 grams to 999 grams; **and** • The neonate died before discharge.
N24	• The applicable MS-DRG 789 – 795 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of 750 grams to 999 grams; **and** • The neonate did not die before discharge.
N37	• The applicable MS-DRG 789 – 795 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of 1,000 grams to 1,499 grams; **and** • The neonate died before discharge.
N38	• The applicable MS-DRG 789 – 795 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of 1,000 grams to 1,499 grams; • The neonate did not die before discharge; **and** • A procedure, other than circumcision or another minor surgery, was performed in the operating room.
N39	• The applicable MS-DRG 789 – 795 criteria were satisfied as outlined in the chart below;

OHCA DRG code	Minimum criteria
	• The neonate had a birth weight of 1,000 grams to 1,499 grams; • The neonate did not die before discharge; **and** • One of the following is applicable: – No procedure was performed in the operating room, or – Only a circumcision or other minor surgery was performed.
N48	• The applicable MS-DRG 789 – 795 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of 1,500 grams to 1,999 grams; **and** • A procedure, other than circumcision or another minor surgery, was performed in the operating room.
N49	• The applicable MS-DRG 789 – 795 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of 1,500 grams to 1,999 grams; **and** • One of the following is applicable: – No procedure was performed in the operating room, or – Only a circumcision or other minor surgery was performed.
N50	• The applicable MS-DRG 789 – 795 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of 2,000 grams to 2,499 grams; **and** • A procedure, other than circumcision or another minor surgery, was performed in the operating room.
N56	• The applicable MS-DRG 790, 791 or 793 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of 2,000 grams to 2,499 grams; **and** • One of the following is applicable: – No procedure was performed in the operating room, or – Only a circumcision or other minor surgery was performed.
N57	• The applicable MS-DRG 792 or 794 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of 2,000 grams to 2,499 grams; **and** • One of the following is applicable: – No procedure was performed in the operating room, or – Only a circumcision or other minor surgery was performed.
N70	• The applicable MS-DRG 789 or 795 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of 2,000 grams to 2,499 grams; **and** • One of the following is applicable: – No procedure was performed in the operating room, or – Only a circumcision or other minor surgery was performed.
N76	• The applicable MS-DRG 790, 791 or 793 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of greater than 2,499 grams; **and** • One of the following is applicable: – No procedure was performed in the operating room, or – Only a circumcision or other minor surgery was performed.
N77	• The applicable MS-DRG 792 or 794 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of greater than 2,499 grams; **and** • One of the following is applicable: – No procedure was performed in the operating room, or – Only a circumcision or other minor surgery was performed.

OHCA DRG code	Minimum criteria
N78	• The applicable MS-DRG 789 or 795 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of greater than 2,499 grams; **and** • One of the following is applicable: – No procedure was performed in the operating room, or – Only a circumcision or other minor surgery was performed.
N80	• The applicable MS-DRG 789 – 795 criteria were satisfied as outlined in the chart below; • The neonate had a birth weight of greater than 2,499 grams; **and** • A procedure, other than circumcision or another minor surgery, was performed in the operating room.

Humana assigns the MS-DRG according to the table below.

MS-DRG code	Minimum criteria
790	A neonate who has a birth weight less than or equal to 999 grams, has completed a gestational age of 23 – 26 weeks or is diagnosed with neonatal RDS (ICD-10-CM diagnosis code P22.0) according to the peer-reviewed literature definition and treatment requirements noted in the Overview section ***Note:*** DRG 790 assignment is intended for critically ill neonates requiring the most complex care **and** prolonged hospital confinement.
791	A neonate who has a birth weight of 1,000 grams to 2,499 grams or has completed a gestational age of only 27 – 36 weeks **and** One or more of the diagnoses on the CMS Major Problems Diagnoses list applicable for the date of service, available in the References section
792	A neonate who has a birth weight of 1,000 grams to 2,499 grams or has completed a gestational age of only 27 – 36 weeks **without** major problems
793	A full-term neonate who has completed a gestational age of 37 weeks or greater **and** One or more of the diagnoses on the CMS Major Problems Diagnoses list, applicable for the date of service, available in the References section
794	A full-term neonate who has completed a gestational age of 37 weeks or greater **and** One or more of the diagnoses on the CMS Significant Problems Diagnoses list, applicable for the date of service, available in the References section
795	A normal neonate not requiring care outside of the newborn nursery
789	A neonate who is transferred to another acute care facility or has died

Humana uses all relevant documentation to review the ICD-10-CM diagnosis codes reported on the claim according to the peer-reviewed literature definition and treatment requirements. If a reported diagnosis code does not satisfy the applicable requirement, it is not considered when calculating the DRG reimbursed.

Humana reviews the level of care as reported by the NICU revenue code according to the table below. If the documentation associated with a charge for a NICU revenue code does not establish that the criteria associated with that revenue code were satisfied, reimbursement for the claim will instead be calculated using the highest NICU revenue code for which the criteria are satisfied.

Nursery level	Revenue code	Minimum criteria
Level 0	0170	Level 0 and Level 1 are considered newborn nursery. Examples include care to observe for: • Sepsis • Temperature instability • Feeding irregularities • Jaundice
Level 1	0171	
Level 2	0172	Neonate who is not severely ill but needs more hours of nursing and medical care than typical neonates, as indicated by **one or more of the following**: • Ventilatory support using one of the following: – Hood oxygen, or – Less than or equal to two liters per minute of oxygen via nasal cannula • Administration of intravenous (IV) heplock medicines • Weaning from nasogastric (NG) or nasojejunal (NJ) tube feedings while trying to increase oral intake, without need for IV fluids • Weaning from isolette or radiant warmer to open crib, without need for temperature support • Documented episodes of apnea, bradycardia or desaturation, for example: – Apnea countdown, – Titrating caffeine or preparing infant for home caffeine administration, or – Preparing infant for home monitor use for apnea, bradycardia or desaturation • Services for neonatal abstinence syndrome (NAS) when the Finnegan score is less than or equal to 8 • Services for neonatal jaundice requiring either: – Use of phototherapy, or – Monitoring of bilirubin levels less frequently than every 12 hours • Supportive care for growing premature infant, or establishing weight gain
Level 3	0173	**Level 2 requirements are met** **and** Temperature control modalities, such as isolette or radiant warmer, are in use **and** Condition requiring 6 to 12 hours of active nursing and medical evaluation, treatment and support, as evidenced by **one or more of the following**: • Ventilatory support using one of the following: – Greater than two liters per minute of oxygen via nasal cannula, – High flow nasal cannula, – Continuous positive airway pressure (CPAP), – Stable chronic mechanical ventilation (on home ventilator settings), or – Conventional ventilator • Umbilical venous catheter (UVC) or invasive cardiopulmonary monitoring (CPM) • Hypoglycemia requiring D-10 bolus therapy • Nothing by mouth (NPO) with either: – All NG feedings, or – Total parenteral nutrition (TPN) • Services for NAS with one of the following: – Three consecutive Finnegan scores greater than or equal to 8, – Two consecutive Finnegan scores greater than or equal to 12, or – Pharmacologic management of NAS

Nursery level	Revenue code	Minimum criteria
Level 4	0174	**Level 3 requirements are met** **and** Other condition that requires ongoing evaluation, active management, or therapy adjustment with near continuous nursing and medical support, as evidenced by **one or more of the following**: • Ventilatory support using high frequency oscillating ventilator (HFOV) • Use of IV bolus therapy, inotropic drugs, chronotropic drugs, calcium channel blockers or IV prostaglandin therapy • Active seizures, with need for anticonvulsants • Surgical repair of severe congenital defect, for example: – Omphalocele repair, – Bowel resection for necrotizing enterocolitis (NEC), or – Tracheoesophageal fistula repair • Inhaled nitric oxide (iNO) • Extracorporeal membrane oxygenation (ECMO) • Therapeutic cooling • Hyperbilirubinemia requiring either: – Exchange transfusion, or – Intravenous immunoglobulin (IVIG) administration • Emergent event (for example, pulmonary arrest or cardiac arrest)

South Carolina Medicaid Payment Policy

To be reimbursed for a medically necessary NICU stay subject to DRG-based pricing, the provider must satisfy the requirements in this policy.

During the NICU stay, the provider must provide all relevant documentation, such as medical records, for utilization management and leveling purposes. Humana uses all relevant documentation to assign the DRG for the case according to the table below. For a case that qualifies for more than one DRG, Humana assigns the DRG associated with the most complex services.

DRG code	Minimum criteria
580	A neonate transferred from any hospital who is less than 5 days old and not born at the current hospital
581	A neonate transferred from any hospital who is less than 5 days old and born at the current hospital
583	A neonate with extracorporeal membrane oxygenation (ECMO)
588	A neonate who has a birth weight less than 1500 grams with a major procedure
589	A neonate who has a birth weight less than 500 grams
591	A neonate who has a birth weight of 500 grams to 749 grams without a major procedure
593	A neonate who has a birth weight of 750 grams to 999 grams without a major procedure
602	A neonate who has a birth weight of 1000 grams to 1249 grams with RDS or another major respiratory condition or major anomaly
603	A neonate who has a birth weight of 1000 grams to 1249 grams with or without another significant condition
607	A neonate who has a birth weight of 1250 grams to 1499 grams with RDS or another major respiratory condition or major anomaly

DRG code	Minimum criteria
608	A neonate who has a birth weight of 1250 grams to 1499 grams with or without other significant condition
609	A neonate who has a birth weight of 1500 grams to 2499 grams with a major procedure
611	A neonate who has a birth weight of 1500 grams to 1999 grams with a major anomaly
612	A neonate who has a birth weight of 1500 grams to 1999 grams with RDS or another major respiratory condition
613	A neonate who has a birth weight of 1500 grams to 1999 grams with a congenital or perinatal infection
614	A neonate who has a birth weight of 1500 grams to 1999 grams with or without another significant condition
621	A neonate who has a birth weight of 2000 grams to 2499 grams with a major anomaly
622	A neonate who has a birth weight of 2000 grams to 2499 grams with RDS or another major respiratory condition
623	A neonate who has a birth weight of 2000 grams to 2499 grams with a congenital or perinatal infection
625	A neonate who has a birth weight of 2000 grams to 2499 grams with another significant condition
626	A normal newborn or neonate who has a birthweight of 2000 grams to 2499 grams with another problem
630	A neonate who has a birth weight greater than 2499 grams with a major cardiovascular procedure
631	A neonate who has a birth weight greater than 2499 grams with another major procedure
633	A neonate who has a birth weight greater than 2499 grams with a major anomaly
634	A neonate who has a birth weight greater than 2499 grams with RDS or another major respiratory condition
636	A neonate who has a birth weight greater than 2499 grams with a congenital or perinatal infection
639	A neonate who has a birth weight greater than 2499 grams with another significant condition
640	A normal newborn or neonate who has a birthweight greater than 2499 grams with another problem
863	Neonatal aftercare

Humana uses all relevant documentation to review the ICD-10-CM diagnosis codes reported on the claim according to the peer-reviewed literature definition and treatment requirements. If a reported diagnosis code does not satisfy the applicable requirement, it is not considered when calculating the DRG reimbursed.

Humana reviews the level of care as reported by the NICU revenue code according to the table below. If the documentation associated with a charge for a NICU revenue code does not establish that the criteria associated with that revenue code were satisfied, reimbursement for the claim will instead be calculated using the highest NICU revenue code for which the criteria are satisfied. Neonates born during a gestational age range of 34 weeks and zero days through 36 weeks and six days are recognized as a high-risk neonatal population and may also require NICU-level care when clinical criteria are met.

Nursery level	Revenue code	Minimum criteria
Level 0	0170	Level 0 and Level 1 are considered newborn nursery. Examples include care to observe for: • Sepsis • Temperature instability • Feeding irregularities • Jaundice
Level 1	0171	
Level 2	0172	Neonate who is not severely ill but needs more hours of nursing and medical care than typical neonates, as indicated by **one or more of the following**: • Ventilatory support using one of the following: – Hood oxygen, or – Less than or equal to two liters per minute of oxygen via nasal cannula • Administration of intravenous (IV) heplock medicines • Weaning from nasogastric (NG) or nasojejunal (NJ) tube feedings while trying to increase oral intake, without need for IV fluids • Weaning from isolette or radiant warmer to open crib, without need for temperature support • Documented episodes of apnea, bradycardia or desaturation, for example: – Apnea countdown, – Titrating caffeine or preparing infant for home caffeine administration, or – Preparing infant for home monitor use for apnea, bradycardia or desaturation • Services for neonatal abstinence syndrome (NAS) when the Finnegan score is less than or equal to 8 • Services for neonatal jaundice requiring either: – Use of phototherapy, or – Monitoring of bilirubin levels less frequently than every 12 hours • Supportive care for growing premature infant, or establishing weight gain
Level 3	0173	**Level 2 requirements are met** **and** Temperature control modalities, such as isolette or radiant warmer, are in use **and** Condition requiring 6 to 12 hours of active nursing and medical evaluation, treatment and support, as evidenced by **one or more of the following**: • Ventilatory support using one of the following: – Greater than two liters per minute of oxygen via nasal cannula, – High flow nasal cannula, – Continuous positive airway pressure (CPAP), – Stable chronic mechanical ventilation (on home ventilator settings), or – Conventional ventilator • Umbilical venous catheter (UVC) or invasive cardiopulmonary monitoring (CPM) • Hypoglycemia requiring D-10 bolus therapy • Nothing by mouth (NPO) with either: – All NG feedings, or – Total parenteral nutrition (TPN) • Services for NAS with one of the following: – Three consecutive Finnegan scores greater than or equal to 8, – Two consecutive Finnegan scores greater than or equal to 12, or – Pharmacologic management of NAS

Nursery level	Revenue code	Minimum criteria
Level 4	0174	**Level 3 requirements are met** **and** Other condition that requires ongoing evaluation, active management, or therapy adjustment with near continuous nursing and medical support, as evidenced by **one or more of the following**: • Ventilatory support using high frequency oscillating ventilator (HFOV) • Use of IV bolus therapy, inotropic drugs, chronotropic drugs, calcium channel blockers or IV prostaglandin therapy • Active seizures, with need for anticonvulsants • Surgical repair of severe congenital defect, for example: – Omphalocele repair, – Bowel resection for necrotizing enterocolitis (NEC), or – Tracheoesophageal fistula repair • Inhaled nitric oxide (iNO) • Extracorporeal membrane oxygenation (ECMO) • Therapeutic cooling • Hyperbilirubinemia requiring either: – Exchange transfusion, or – Intravenous immunoglobulin (IVIG) administration • Emergent event (for example, pulmonary arrest or cardiac arrest)

Virginia Medicaid Payment Policy

To be reimbursed for a medically necessary NICU stay subject to DRG-based pricing, the provider must satisfy the requirements in this policy.

During the NICU stay, the provider must provide all relevant documentation, such as medical records, for utilization management and leveling purposes. Humana uses all relevant documentation to assign the DRG for the case according to the table below. For a case that qualifies for more than one DRG, Humana assigns the DRG associated with the most complex services.

DRG code	Minimum criteria
580	A neonate transferred from any hospital who is less than 5 days old and not born at the current hospital
581	A neonate transferred from any hospital who is less than 5 days old and born at the current hospital
583	A neonate with extracorporeal membrane oxygenation (ECMO)
588	A neonate who has a birth weight less than 1500 grams with a major procedure
589	A neonate who has a birth weight less than 500 grams, or a birth weight of 500 grams to 999 grams and a gestational age of less than 24 weeks, or a birth weight of 500 grams to 749 grams with a major anomaly or without life sustaining intervention
591	A neonate who has a birth weight of 500 grams to 749 grams without a major procedure
593	A neonate who has a birth weight of 750 grams to 999 grams without a major procedure
602	A neonate who has a birth weight of 1000 grams to 1249 grams with RDS or another major respiratory condition or major anomaly
603	A neonate who has a birth weight of 1000 grams to 1249 grams with or without another significant condition

DRG code	Minimum criteria
607	A neonate who has a birth weight of 1250 grams to 1499 grams with RDS or another major respiratory condition or major anomaly
608	A neonate who has a birth weight of 1250 grams to 1499 grams with or without other significant condition
609	A neonate who has a birth weight of 1500 grams to 2499 grams with a major procedure
611	A neonate who has a birth weight of 1500 grams to 1999 grams with a major anomaly
612	A neonate who has a birth weight of 1500 grams to 1999 grams with RDS or another major respiratory condition
613	A neonate who has a birth weight of 1500 grams to 1999 grams with a congenital or perinatal infection
614	A neonate who has a birth weight of 1500 grams to 1999 grams with or without another significant condition
621	A neonate who has a birth weight of 2000 grams to 2499 grams with a major anomaly
622	A neonate who has a birth weight of 2000 grams to 2499 grams with RDS or another major respiratory condition
623	A neonate who has a birth weight of 2000 grams to 2499 grams with a congenital or perinatal infection
625	A neonate who has a birth weight of 2000 grams to 2499 grams with another significant condition
626	A normal newborn or neonate who has a birthweight of 2000 grams to 2499 grams with another problem
630	A neonate who has a birth weight greater than 2499 grams with a major cardiovascular procedure
631	A neonate who has a birth weight greater than 2499 grams with another major procedure
633	A neonate who has a birth weight greater than 2499 grams with a major anomaly
634	A neonate who has a birth weight greater than 2499 grams with RDS or another major respiratory condition
636	A neonate who has a birth weight greater than 2499 grams with a congenital or perinatal infection
639	A neonate who has a birth weight greater than 2499 grams with another significant condition
640	A normal newborn or neonate who has a birthweight greater than 2499 grams with another problem
863	Neonatal aftercare

Humana uses all relevant documentation to review the ICD-10-CM diagnosis codes reported on the claim according to the peer-reviewed literature definition and treatment requirements. If a reported diagnosis code does not satisfy the applicable requirement, it is not considered when calculating the DRG reimbursed.

Humana reviews the level of care as reported by the NICU revenue code according to the table below. If the documentation associated with a charge for a NICU revenue code does not establish that the criteria associated with that revenue code were satisfied, reimbursement for the claim will instead be calculated using the highest NICU revenue code for which the criteria are satisfied.

Nursery level	Revenue code	Minimum criteria
Level 0	0170	Level 0 and Level 1 are considered newborn nursery. Examples include care to observe for: • Sepsis • Temperature instability • Feeding irregularities • Jaundice
Level 1	0171	
Level 2	0172	Neonate who is not severely ill but needs more hours of nursing and medical care than typical neonates, as indicated by **one or more of the following**: • Ventilatory support using one of the following: – Hood oxygen, or – Less than or equal to two liters per minute of oxygen via nasal cannula • Administration of intravenous (IV) heplock medicines • Weaning from nasogastric (NG) or nasojejunal (NJ) tube feedings while trying to increase oral intake, without need for IV fluids • Weaning from isolette or radiant warmer to open crib, without need for temperature support • Documented episodes of apnea, bradycardia or desaturation, for example: – Apnea countdown, – Titrating caffeine or preparing infant for home caffeine administration, or – Preparing infant for home monitor use for apnea, bradycardia or desaturation • Services for neonatal abstinence syndrome (NAS) when the Finnegan score is less than or equal to 8 • Services for neonatal jaundice requiring either: – Use of phototherapy, or – Monitoring of bilirubin levels less frequently than every 12 hours • Supportive care for growing premature infant, or establishing weight gain
Level 3	0173	**Level 2 requirements are met** **and** Temperature control modalities, such as isolette or radiant warmer, are in use **and** Condition requiring 6 to 12 hours of active nursing and medical evaluation, treatment and support, as evidenced by **one or more of the following**: • Ventilatory support using one of the following: – Greater than two liters per minute of oxygen via nasal cannula, – High flow nasal cannula, – Continuous positive airway pressure (CPAP), – Stable chronic mechanical ventilation (on home ventilator settings), or – Conventional ventilator • Umbilical venous catheter (UVC) or invasive cardiopulmonary monitoring (CPM) • Hypoglycemia requiring D-10 bolus therapy • Nothing by mouth (NPO) with either: – All NG feedings, or – Total parenteral nutrition (TPN) • Services for NAS with one of the following: – Three consecutive Finnegan scores greater than or equal to 8, – Two consecutive Finnegan scores greater than or equal to 12, or – Pharmacologic management of NAS

Nursery level	Revenue code	Minimum criteria
Level 4	0174	**Level 3 requirements are met** **and** Other condition that requires ongoing evaluation, active management, or therapy adjustment with near continuous nursing and medical support, as evidenced by **one or more of the following**: • Ventilatory support using high frequency oscillating ventilator (HFOV) • Use of IV bolus therapy, inotropic drugs, chronotropic drugs, calcium channel blockers or IV prostaglandin therapy • Active seizures, with need for anticonvulsants • Surgical repair of severe congenital defect, for example: – Omphalocele repair, – Bowel resection for necrotizing enterocolitis (NEC), or – Tracheoesophageal fistula repair • Inhaled nitric oxide (iNO) • Extracorporeal membrane oxygenation (ECMO) • Therapeutic cooling • Hyperbilirubinemia requiring either: – Exchange transfusion, or – Intravenous immunoglobulin (IVIG) administration • Emergent event (for example, pulmonary arrest or cardiac arrest)

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General Humana Resources

- Availity – Providers can register for access to information on a variety of topics such as eligibility, benefits, referrals, authorizations, claims and electronic remittances.
- Claims processing edit notifications – Alerts of upcoming claims payment changes are posted on the first Friday of each month.
- Claims resources – Providers can find information on referrals, authorizations, electronic claim submissions and more.
- Making it easier – This page contains an educational series for providers and healthcare professionals.
- Medical and pharmacy coverage policies – Humana publishes determinations of coverage of medical procedures, devices and medications for the treatment of various conditions. There may be variances in coverage among plans.
- Publications – This page can help you find our quarterly newsletter, provider manual and other resources to help you do business with us.

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